

C810

Foundations in Healthcare Data Management

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A quality analyst at a 12-hospital system that uses two different EHR vendors pulls data from the state trauma registry to compare patient outcomes. The registry was funded by a federal grant. What type of data source is the registry?

- A. The legal health record.
- B. A primary data source, because it contains patient-level data.
- C. A primary source, because it is government funded.
- D. A secondary data source.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which of these is a primary data source?

- A. The hospital's disease index.
- B. A national quality benchmark database.
- C. A patient's health record documenting care at the hospital.
- D. A research data set built from many patient records.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A patient asks a hospital's release-of-information office, which recently moved to a new building, for a copy of her billing records along with her medical record. The ROI clerk, who was hired last month, says billing records are not part of the medical record. Under HIPAA, does she have a right to access the billing records?

- A. No; the right of access covers only the legal health record.
- B. Yes, but only through a court subpoena.
- C. No; patients may see only notes written by physicians.
- D. Yes; billing records are part of the designated record set, which the right of access covers.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A cardiologist at a hospital with a strong teaching reputation requests the full record of a patient she is about to treat, including notes from an unrelated admission five years ago. The HIM department has a backlog this week. Does the minimum necessary standard limit this disclosure?

- A. No; disclosures to a provider for treatment are exempt from minimum necessary.
- B. No, because the backlog justifies releasing everything at once.
- C. Yes; only the current admission may be released.
- D. Yes; records older than five years cannot be shared.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

A health plan asks a hospital for records to process a claim for a patient's stay. Which rule governs how much to release?

- A. Release nothing without the patient's written authorization.
- B. Release the entire record, as for treatment.
- C. De-identify the record before any release.
- D. Minimum necessary applies; release only what the plan needs to process the claim.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A health system board, which meets quarterly in a renovated boardroom, wants clear accountability for how "active patient" is defined across its EHR, billing, and analytics systems. Three departments now use three different definitions. The IT department recently hired 10 new analysts. What is the best first step?

- A. Have IT choose a definition and hard-code it.
- B. Set up data governance that assigns decision rights and a data steward to own the definition.
- C. Let each department keep its own definition, since definitions should always follow local workflow.
- D. Assign the 10 new analysts to reconcile the reports every month.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A 200-bed hospital that recently won a patient-experience award finds that pathology results reach the EHR three days after they are finalized, often after physicians have already made discharge decisions. The results themselves are correct and complete. Which data quality characteristic is the problem?

- A. Granularity.
- B. Accuracy.
- C. Timeliness.
- D. Completeness.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A hospital's retention schedule, approved by its legal counsel in 2019, calls for destroying certain adult records after 10 years. A batch reaches that age this month, but one record belongs to a patient whose family has filed a lawsuit against the hospital. The shredding vendor offers a discount for large batches. What should HIM do with that record?

- A. Place it on legal hold and keep it until counsel releases the hold.
- B. Destroy the whole batch now to get the vendor discount.
- C. Keep every record permanently from now on.
- D. Destroy it with the batch, following the schedule.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A hospital's lab system, purchased from a vendor based in another state, must send results to the EHR so that a hemoglobin A1c test is identified the same way in both systems. The interface team works a night shift. Which standard names the lab test itself?

- A. SNOMED CT.
- B. ICD-10-CM.
- C. LOINC.
- D. HL7 FHIR.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

Two systems already use LOINC for lab test names but still cannot pass results to each other. What kind of standard do they still need?

- A.** A messaging or exchange standard such as HL7 FHIR.
- B.** ICD-10-PCS.
- C.** SNOMED CT.
- D.** Nothing; a shared terminology alone guarantees exchange.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

At a hospital that registers about 400 patients a day, a registration clerk working a double shift searches for a returning patient, does not find her because her last name was misspelled, and creates a new medical record number. Which MPI problem has occurred?

- A.** An overlap.
- B.** A duplicate record.
- C.** No problem, as long as staff use the new number from now on.
- D.** An overlay.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

A clinician opens a chart and finds another patient's lab results filed under the first patient's medical record number. Which MPI error is this?

- A.** A duplicate record.
- B.** An overlay.
- C.** An overlap.
- D.** A transcription error.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A quality analyst at a 12-hospital system ~~that uses two different EHR vendors~~ pulls data from the state trauma registry to compare patient outcomes. ~~The registry was funded by a federal grant.~~ What type of data source is the registry?

Target word: classify the data source **Decides:** The registry is abstracted from patient records for purposes other than direct care.

A	Adjacent category	The legal health record is the provider's business record of care, not a registry.
B	Confusion set	Patient-level detail does not make a source primary; origin in direct care does.
C	Camouflage	Funding source does not change how the data was created.
D	Correct	Registries aggregate data abstracted from primary records for research, quality, and public health.

2. Answer C Concept 1 • transfer

Which of these is a primary data source?

Target word: identify primary **Decides:** Created during direct patient care.

A	Confusion set	Indexes are built from records, so they are secondary.
B	Adjacent category	A comparative database is aggregated secondary data.
C	Correct	The record created during care is the primary source.
D	Inverted operation	Built from records, so it runs the wrong direction for primary.

3. Answer D Concept 2

A patient asks a hospital's release-of-information office, ~~which recently moved to a new building,~~ for a copy of her billing records along with her medical record. The ROI clerk, ~~who was hired last month,~~ says billing records are not part of the medical record. Under HIPAA, does she have a right to access the billing records?

Target word: scope of right of access **Decides:** Billing records are part of the designated record set.

A	Confusion set	Confuses the legal health record with the designated record set.
B	Adjacent category	Subpoenas govern third-party legal requests, not a patient's own access.
C	Absolute language	Access is not limited to physician documentation.
D	Correct	HIPAA's designated record set includes billing records used to make decisions about the individual.

4. Answer A Concept 3

A cardiologist at a hospital ~~with a strong teaching reputation~~ requests the full record of a patient she is about to treat, including notes from an unrelated admission five years ago. ~~The HIM department has a backlog this week.~~ Does the minimum necessary standard limit this disclosure?

Target word: does minimum necessary apply **Decides:** Disclosure to a provider for treatment.

A	Correct	HIPAA exempts treatment disclosures from the minimum necessary standard.
B	Camouflage	Right answer for the wrong reason; workload is not the basis.
C	Dropped element	Drops the treatment exception.
D	Wrong stem number	Record age does not decide minimum necessary.

5. Answer D Concept 3 · transfer

A health plan asks a hospital for records to process a claim for a patient's stay. Which rule governs how much to release?

Target word: minimum necessary for payment **Decides:** Payment is not a treatment disclosure.

A	Absolute language	Payment is a permitted use and disclosure without authorization.
B	Inverted operation	Applies the treatment exception to a payment request.
C	Adjacent category	The plan needs identifiable data to pay the claim; de-identification is for other uses.
D	Correct	Payment disclosures are permitted but limited to the minimum necessary.

6. Answer B Concept 4

A health system board, ~~which meets quarterly in a renovated boardroom,~~ wants clear accountability for how "active patient" is defined across its EHR, billing, and analytics systems. Three departments now use three different definitions. ~~The IT department recently hired 10 new analysts.~~ What is the best first step?

Target word: who decides data definitions **Decides:** No one owns the definition; decision rights are missing.

A	Adjacent category	That is data management acting without governance or business ownership.
B	Correct	Governance establishes accountability and a single approved definition.
C	Absolute language	Conflicting definitions are the problem governance exists to fix.
D	Camouflage	Reacts to the new-hire detail and treats the symptom, not the missing ownership.

7. Answer C Concept 5

A 200-bed hospital ~~that recently won a patient experience award~~ finds that pathology results reach the EHR three days after they are finalized, often after physicians have already made discharge decisions. The results themselves are correct and complete. Which data quality characteristic is the problem?

Target word: name the quality characteristic **Decides:** Correct, complete data arriving too late to use.

A	Adjacent category	Granularity is level of detail, not delivery time.
B	Confusion set	The stem says the results are correct.
C	Correct	Data must be available when it is needed for decisions.
D	Dropped element	Ignores the stem's statement that the results are complete.

8. Answer A Concept 6

A hospital's retention schedule, ~~approved by its legal counsel in 2019,~~ calls for destroying certain adult records after 10 years. A batch reaches that age this month, but one record belongs to a patient whose family has filed a lawsuit against the hospital. ~~The shredding vendor offers a discount for large batches.~~ What should HIM do with that record?

Target word: destruction vs. legal hold **Decides:** Pending litigation involving the record.

A	Correct	A legal hold suspends normal destruction for records relevant to litigation.
B	Camouflage	Vendor pricing never overrides a legal hold.
C	Absolute language	The hold applies to relevant records; the schedule still governs the rest.
D	Dropped element	Ignores the lawsuit; destroying relevant records can be spoliation of evidence.

9. Answer C Concept 7

A hospital's lab system, ~~purchased from a vendor based in another state~~, must send results to the EHR so that a hemoglobin A1c test is identified the same way in both systems. ~~The interface team works a night shift.~~ Which standard names the lab test itself?

Target word: standard for naming lab tests **Decides:** Identify the test (observation) consistently.

A	Confusion set	SNOMED CT codes clinical findings and concepts, not the lab test order or result name.
B	Adjacent category	ICD-10-CM classifies diagnoses for reporting and billing.
C	Correct	LOINC provides standard names and codes for lab tests and observations.
D	Confusion set	FHIR carries the message between systems; it does not name the test.

10. Answer A Concept 7 · transfer

Two systems already use LOINC for lab test names but still cannot pass results to each other. What kind of standard do they still need?

Target word: the other half of interoperability **Decides:** Shared vocabulary exists; exchange format does not.

A	Correct	Exchange standards define how data is packaged and sent.
B	Adjacent category	A procedure classification does not move data between systems.
C	Confusion set	Another terminology does not create an exchange method.
D	Absolute language	Terminology and messaging are separate layers of interoperability.

11. Answer B Concept 8

At a hospital ~~that registers about 400 patients a day~~, a registration clerk ~~working a double shift~~ searches for a returning patient, does not find her because her last name was misspelled, and creates a new medical record number. Which MPI problem has occurred?

Target word: name the MPI error **Decides:** One person now has two record numbers.

A	Adjacent category	An overlap is one person with different numbers across facilities in an enterprise.
B	Correct	One patient with two MRNs in the same facility is a duplicate.
C	Dropped element	The old record's history is split off, creating a safety risk; duplicates must be merged.
D	Confusion set	An overlay is two people sharing one record number.

12. Answer B Concept 8 · transfer

A clinician opens a chart and finds another patient's lab results filed under the first patient's medical record number. Which MPI error is this?

Target word: name the MPI error — reversed **Decides:** Two people under one record number.

A	Inverted operation	A duplicate is the reverse: one person with two numbers.
B	Correct	Two patients' information under one MRN is an overlay.
C	Confusion set	Overlap involves multiple facilities, one person.
D	Adjacent category	Transcription errors are wording mistakes in dictated notes, not identity errors.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	10	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	3	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	9	
Camouflage	Answers a stem fact that was never the question.	4	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Primary vs. secondary data sources	The patient health record created during care is the primary source; registries, indexes, and databases abstracted from records for other uses are secondary sources.	Calling any patient-level data set “primary.”	Was the data created in direct care, or pulled from records for another purpose?
2	Legal health record vs. designated record set	The designated record set (HIPAA) includes medical and billing records used to make decisions about the individual, and the patient’s right of access covers it.	Limiting patient access to the legal health record only.	A patient asks for billing or other non-chart records.
3	Minimum necessary	Use and disclose only the minimum PHI needed, except for treatment, disclosures to the patient, authorized disclosures, and a few others.	Applying minimum necessary to a provider who is treating the patient.	Who is asking and why: treatment vs. payment or operations.
4	Data governance	Governance assigns decision rights and accountability (policies, stewards, definitions); data management carries out those decisions.	Letting IT or each department settle definitions on its own.	Conflicting definitions across departments and no owner.
5	Data quality characteristics	Each characteristic names a different failure: accuracy (correct), completeness, consistency, timeliness (available when needed), granularity, relevancy, and more.	Calling every data problem “accuracy.”	The stem rules out some failures (“correct and complete”) and describes one.
6	Retention, destruction, and legal hold	Records follow the retention schedule unless a legal hold applies; a hold suspends destruction until counsel releases it.	Destroying on schedule despite pending litigation.	A record due for destruction that is tied to a lawsuit or investigation.
7	Terminology vs. messaging standards	LOINC names lab tests and observations; SNOMED CT codes clinical terms; ICD classifies diagnoses; HL7/FHIR defines how data is exchanged between systems.	Expecting one vocabulary to make systems exchange data, or using a billing classification as a clinical terminology.	“Identify the test the same way” vs. “send the result between systems.”
8	Master patient index errors	Duplicate = one person with two record numbers; overlay = two people under one record number; overlap = one person with different numbers across facilities in an enterprise.	Mixing up duplicate and overlay.	Count the people and count the record numbers.

Course scope (public description): Foundations in Healthcare Data Management introduces students to the concepts and terminology used in health data and health information management. This course teaches students

how to apply data management and governance principles in the healthcare environment. The student will learn about electronic health records (EHR), legal considerations, information governance, data management, health information management (HIM), and secondary data sources. In addition to the e-text and numerous additional articles and video resources, the student will engage with case studies and knowledge checks to assist with learning. There are no prerequisites for this course.